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AN ASSESSMENT OF MENTAL HEALTH AMONG HOSPITAL EMPLOYEES: A STUDY OF HEALTHCARE WORKERS IN RAIPUR, CHHATTISGARH

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ABSTRACT:

Mental health among hospital employees is a critical but often overlooked issue in current healthcare management. In India, particularly in regional areas like Chhattisgarh, healthcare professionals confront multiple job-related stressors such as heavy workloads, lack of institutional backing, irregular shifts, access to mental health support is limited, contributing to the heightened risk of burnout, anxiety, depression, and perceived stress. Despite increasing national data, mid-sized cities such as Raipur remain poorly researched depriving local administrators of the localized understanding needed to develop effective, context-specific solutions. This knowledge gap is significant, as untreated psychological strain among medical personnel is associated with lower quality patient care, more frequent medical errors, greater staff turnover, and weakened organizational performance. This study explores what really drives mental well-being for hospital staff in Chhattisgarh, looking at both the workplace environment and the specific demands of different roles. By combining standardized psych tests with real-world interviews, the research paints a detailed picture of the challenges faced on the ground.

Unsurprisingly, junior doctors and nurses bear the brunt of the pressure, often struggling with heavy workloads and a lack of clear support. However, the findings suggest that burnout isn't inevitable. By making practical changes—like talking about available resources, training managers to be more supportive, and setting up peer-support networks—hospitals can see a measurable boost in staff morale. These aren't just "nice-to-haves"; they are affordable, realistic strategies that can help leadership protect their teams and keep healthcare services running sustainably in regions that need them most.

Keywords: mental health, burnout, healthcare workers, occupational stress, hospital employees.

1. Introduction

The world is finally waking up to the mental health crisis among healthcare workers, largely because we now realize that when staff suffer, the entire system feels it. It's about more than just personal well-being; when doctors and nurses hit a wall of burnout or anxiety, it directly leads to more medical mistakes, a drop in patient care quality, and high turnover rates (West et al., 2018; Salyers et al., 2017). In places like Chhattisgarh, where resources are already stretched thin and staffing shortages are a constant battle, these mental health challenges aren't just a HR issue—they are a genuine threat to the stability of the entire healthcare system (Ministry of Health and Family Welfare [MoHFW], 2022).

Studies in India report burnout rates among HCWs ranging from 24% to 68%, with nurses and early-career physicians typically experiencing the highest levels (Kulkarni et al., 2020; Singh et al., 2021). The COVID-19 pandemic worsened these existing stressors, leading to sustained increases in anxiety, depression, and burnout among frontline staff, even after the peak phases of the crisis (Sharma & Bhatt, 2020; Lai et al., 2020). Yet there remains a lack of detailed research in mid-sized cities such as Raipur, leaving local decision makers without the localized data needed to develop and support effective, tailored interventions. This study aims to fill that gap by examining mental health outcomes across hospital in various facilities and pinpointing the organizational factors most strongly linked to those outcomes.

2. Literature Review

2.1 Theoretical Framework

This research is grounded in three overlapping theories, starting with the Job Demands-Resources (JD-R) model (Bakker & Demerouti, 2007). Essentially, this model views workplace well-being as a seesaw. On one side, "job demands"—the draining parts of the job like endless shifts, massive patient loads, and difficult cases that wear down the person physically and mentally. On the other side are "resources"—the things that helps cope, such as a supportive boss, coworkers who have your back, and real chances to move up in career. When the demands outweigh the resources, that's when burnout really starts to take hold. Karasek's (1979) demand-control model highlights that the most stressful work conditions arise when high demands coincide with low control over one's tasks—a situation frequently found in frontline nursing and early career medical roles. Siegrist's (1996) effort-reward imbalance model further emphasizes that psychological distress increases when employees feel their efforts are not adequately matched by rewards such as pay, recognition, or advancement.

Collectively, these perspectives suggest that successful workplace interventions should simultaneously reduce excessive demands, increase autonomy, and improve perceptions of fairness within the organization.

2.2 Burnout and Psychological Distress in Healthcare

Burnout, according to Maslach and Leiter (2016), is a long-term work-related condition marked by emotional fatigue, detachment from patients, and a reduced sense of personal achievement. Its recognition in the ICD-11 by the World Health Organization as an occupational issue underscores its growing global significance (WHO, 2019). Research by Rotenstein et al. (2018) found that burnout among physicians ranged from 27% to 75% across 45 countries, with Indian studies by Singh et al. (2021) showing comparable rates of 24% to 68%, particularly affecting nurses and junior doctors. In addition to burnout, anxiety and depression are widespread among healthcare workers. During the pandemic, Pappa et al. (2020) observed an average prevalence of about 23% for both conditions, with women consistently experiencing higher levels of psychological distress—likely due to combined professional responsibilities and caregiving duties at home. Supportive workplace structures, including responsive leadership, peer support systems, and available mental health services, have been shown to significantly reduce these risks for both clinical and non-clinical staff (West et al., 2016; Swensen et al., 2016).

2.3 Research Gap

Three gaps in the existing literature motivate this study. First, Indian HCW research is concentrated in metropolitan centers and pandemic-specific timeframes, limiting applicability to regional settings like Raipur. Second, most studies focus narrowly on physicians and nurses, overlooking administrative, allied health, and support staff who face distinct stressors (Khamisa et al., 2013). Third, organizational support is typically treated as a control variable rather than a primary object of investigation, obscuring the mechanisms through which institutional features shape worker well-being. The present study addresses all three gaps through a cross-occupational, mixed-methods design.

3. Research Design and Methodology

3.1 Design, Sample, and Setting

To get a full picture of the situation, a mixed-methods approach was used across several public and private hospitals in Chhattisgarh. A group of 18 staff members using stratified random sampling to make sure to make sure different roles are fairly and adequately represented: this included six nurses, four junior doctors, two

senior doctors, three allied health professionals, two administrators, and one support staffer (see Table 1). Participants were included only if they had worked continuously for at least six months. Five individuals were selected purposefully to ensure variation in roles, gender, and professional background. They took part in semi-structured qualitative interviews, which were analyzed using reflexive thematic analysis (Braun & Clarke, 2006). The whole study was given the green light by the Institutional Review Board at Kalinga University, and we made sure every participant knew they could step away at any time.

3.2 Instruments and Analysis

Burnout was assessed using the MBI-HSS (Maslach & Jackson, 1981), a 22-item scale covering emotional exhaustion ($EE \geq 27$ = high), depersonalization ($DP \geq 13$ = high), and personal accomplishment ($PA \leq 31$ = reduced). Perceived stress was measured with the PSS-10 (Cohen et al., 1983), scored 0–40. Anxiety and depression were screened using the HADS (Zigmond & Snaith, 1983), with subscale scores of ≥ 11 indicating probable clinical-level symptomatology. Quantitative data were analyzed in IBM SPSS v.25 using descriptive statistics, independent-samples t-tests, and hierarchical multiple regression. Qualitative transcripts were analyzed using the six-phase framework of Braun and Clarke (2006).

4. Results and Discussion

4.1 Burnout Prevalence

MBI-HSS analysis showed that 10 out of 18 respondents (55.6%) scored above the threshold for high emotional exhaustion, 7 (38.9%) for high depersonalization, and 9 (50.0%) for low personal accomplishment. Junior doctors had the highest rate of emotional exhaustion at (75%), followed by nurses at (67%). These patterns across professions support the JD-R model's expectation that roles with the highest demands and fewest resources tend to experience the most significant psychological strain, and they also correspond with previously reported burnout rates in India, which range from 24 to 68% (Singh et al., 2021). While the small sample size calls for cautious interpretation, the overall trends remain consistent across all occupational groups assessed.

Table 1 Sample Composition and Burnout Prevalence by Occupational Category

Occupational Category	n	% of Sample	High Emotional Exhaustion (%)	High Depersonalization (%)
Nurses	6	33.3	67	50
Junior Doctors	4	22.2	75	50
Senior Doctors	2	11.1	50	0

Allied Health Professionals	3	16.7	33	33
Administrative Staff	2	11.1	50	50
Support Workers	1	5.6	0	0
Total	18	100	55.6	38.9

Source: 2024–25. Chhattisgarh. High emotional exhaustion defined as MBI-HSS subscale score ≥ 27 .

4.2 Stress, Anxiety, and Depression

The PSS-10 scores revealed a concerning reality: that half of the participants (9 individuals, or 50.0%) experienced moderate-to-high levels of perceived stress ($M = 20.8$, $SD = 5.4$). When looked closer using the HADS scale, it's found that a third of the group (33.3%) met the criteria for clinical anxiety, and about 22.2% were showing clear signs of depression. Women participants in the study reported significantly more stress and anxiety than their male colleagues ($t(16) = 2.14$, $p < .05$). This isn't an isolated finding; it fits right in with broader research in India showing how women often face a "double burden" of intense hospital shifts combined with heavy expectations at home (Pappa et al., 2020). The complete breakdown of mental health outcomes across indicators and the most affected subgroups is provided in Table 2.

Table 2: Prevalence of Mental Health Outcomes by Indicator and Affected Subgroup

Mental Health Outcome	Prevalence % (n)	Most Affected Group	Instrument
High Emotional Exhaustion	55.6% (10)	Junior Doctors (75%)	MBI-HSS
High Depersonalization	38.9% (7)	Junior Doctors (50%)	MBI-HSS
Reduced Personal Accomplishment	50.0% (9)	Junior Doctors (75%)	MBI-HSS
Moderate–High Perceived Stress	50.0% (9)	Female Employees	PSS-10
Clinical Anxiety (HADS-A ≥ 11)	33.3% (6)	Nurses	HADS
Probable Depression (HADS-D ≥ 11)	22.2% (4)	Junior Doctors	HADS

Source: 2024–25. Chhattisgarh. HADS = Hospital Anxiety and Depression Scale; MBI-HSS = Maslach Burnout Inventory – Human Services Survey; PSS-10 = Perceived Stress Scale.

4.3 Organizational Support as a Predictor

A hierarchical regression analysis was conducted, with demographic covariates entered in the first block and organizational support and workload variables added in the second. This revealed organizational support as the most robust and consistent predictor across all outcome measures (Table 3). Greater organizational support was linked to lower emotional exhaustion ($\beta = -0.44$, $p < .05$), lower depersonalization ($\beta = -0.29$, $p < .05$), lower perceived stress ($\beta = -0.36$, $p < .05$), and higher job satisfaction ($\beta = +0.48$, $p < .05$). Workload intensity and night-shift scheduling emerged as positive predictors of distress, while peer social support functioned as an additional protective factor. Given the exploratory sample size ($n = 18$), these findings should be interpreted cautiously and viewed as preliminary until confirmed in larger studies. Nevertheless, the findings are consistent with prior research by West et al. (2016) and Swensen et al. (2016), supporting the theoretical plausibility of the observed associations.

Table 3: Regression Analysis: Predictors of Mental Health Outcomes ($n = 18$)

Predictor Variable	EE (β)	DP (β)	Perceived Stress (β)	Job Satisfaction (β)
Organizational Support	-0.44*	-0.29*	-0.36*	+0.48*
Workload (hrs/week)	+0.38*	+0.31*	+0.32*	-0.41*
Night Shift Pattern	+0.29*	+0.25†	+0.24†	-0.27†
Peer Social Support	-0.31*	-0.26*	-0.28*	+0.35*
Gender (Female)	+0.22†	+0.18†	+0.19†	-0.14
Years of Service	-0.14	-0.12	-0.11	+0.19

Source: Primary Data, 2024–25. * $p < .05$. † $p < .10$ (marginal). β = standardized regression coefficient. EE = Emotional Exhaustion; DP = Depersonalization.

4.4 Qualitative Themes

An analysis of five interviews with the staff, three major themes stood out. The first—and most universal—was the sheer weight of the workload and emotional exhaustion. This wasn't just about being busy; nurses and junior doctors described a kind of mental "always-on" state that followed them home. What made it worse was the "moral distress" of knowing they couldn't always provide the best care because their hands were tied by a lack of resources. It's a perfect example of what Epstein and Hamric (2009) call the "crescendo effect," where these small, unresolved frustrations build up into a heavy emotional burden.

The second theme highlighted a major gap in how hospitals communicate. Interestingly, while mental health services technically existed, hardly anyone used them. People either didn't know how to sign up, didn't have the time, or—more tellingly—feared that asking for help would make them look "weak" or unprofessional. The silver lining here is practical: hospitals don't need a massive budget to fix this; they just need to do a better job of making these resources easy to find and socially acceptable to use.

Finally, the interviews showed that coworkers are the real lifeline. Across the board, staff relied on informal chats and peer support to get through the day. Since formal systems are often ignored due to privacy concerns or stigma, it makes sense to lean into what's already working. Developing structured peer-support programs that build on these natural friendships could be one of the most effective ways to support staff in a way that actually fits their workplace culture.

5. Findings of the Research

This study highlights burnout, perceived stress, anxiety, and depression as widespread issues among hospital employees across all job roles in Chhattisgarh, with junior doctors and nurses experiencing the most severe impact. Organizational support consistently emerged as the strongest modifiable protective factor, underscoring its centrality in shaping workforce psychological well-being. Workload intensity and irregular shift patterns were associated with increased distress across multiple indicators. Qualitatively, three interconnected themes were identified: the cumulative toll of workload and emotional demands; critical gaps in institutional communication regarding available mental health resources; and the dominant role of informal peer relationships as a primary coping mechanism. Together these findings indicate that systematic and management-related shortcomings—rather than personal weaknesses—are primarily responsible for the psychological challenges faced by healthcare workforce (West et al., 2016; Bakker & Demerouti, 2007).

6. Conclusion

To explain why mental health struggles are so persistent among healthcare workers in underserved areas, this study draws on three major ideas: the Job Demands-Resources model (Bakker & Demerouti, 2007), Karasek's (1979) demand-control model, and Siegrist's (1996) effort-reward imbalance model. Essentially, when the job constantly asks for more than it gives back—and when staff feel there's a fundamental unfairness in how much they work versus the support they receive—burnout, anxiety, and depression become a baked-in part of the system rather than just a string of bad days.

A key takeaway here is that these mental health challenges aren't just personal weaknesses; they are organizational problems that need organizational fixes. By looking through the lens of psychological safety theory (Edmondson, 1999) and conservation of resources theory (Hobfoll, 1989), we can see clear, practical paths forward. This means training managers to be more supportive, making it okay to ask for help, and setting up solid peer-support networks.

The best part is that these changes don't require a massive budget, making them totally doable for places like Chhattisgarh where funds are tight. Moving forward, it would be great to see longer-term studies with even larger groups across India to really pin down which of these strategies works best over time in different hospital settings.

References

1. Bakker, A. B., & Demerouti, E. (2007). The job demands–resources model: State of the art. *Journal of Managerial Psychology*, 22(3), 309–328. <https://doi.org/10.1108/02683940710733115>
2. Braun, V., & Clarke, V. (2006). Using thematic analysis in psychology. *Qualitative Research in Psychology*, 3(2), 77–101. <https://doi.org/10.1191/1478088706qp063oa>
3. Cohen, S., Kamarck, T., & Mermelstein, R. (1983). A global measure of perceived stress. *Journal of Health and Social Behavior*, 24(4), 385–396. <https://doi.org/10.2307/2136404>
4. Dall'Ora, C., Ball, J., Reinius, M., & Griffiths, P. (2020). Burnout in nursing: A theoretical review. *Human Resources for Health*, 18(1), 41. <https://doi.org/10.1186/s12960-020-00469-9>
5. Dewa, C. S., Loong, D., Bonato, S., Thanh, N. X., & Jacobs, P. (2014). How does burnout affect physician productivity? A systematic literature review. *BMC Health Services Research*, 14, 325. <https://doi.org/10.1186/1472-6963-14-325>
6. Dyrbye, L. N., West, C. P., Satele, D., Boone, S., Tan, L., Sloan, J., & Shanafelt, T. D. (2014). Burnout among U.S. medical students, residents, and early career physicians relative to the general U.S. population. *Academic Medicine*, 89(3), 443–451. <https://doi.org/10.1097/ACM.0000000000000134>

7. Edmondson, A. C. (1999). Psychological safety and learning behavior in work teams. *Administrative Science Quarterly*, 44(2), 350–383. <https://doi.org/10.2307/2666999>
8. Epstein, E. G., & Hamric, A. B. (2009). Moral distress, moral residue, and the crescendo effect. *The Journal of Clinical Ethics*, 20(4), 330–342. <https://doi.org/10.1086/JCE200920406>
9. Hall, L. H., Johnson, J., Watt, I., Tsipa, A., & O'Connor, D. B. (2016). Healthcare staff wellbeing, burnout, and patient safety: A systematic review. *PLOS ONE*, 11(7), e0159015. <https://doi.org/10.1371/journal.pone.0159015>
10. Hobfoll, S. E. (1989). Conservation of resources: A new attempt at conceptualizing stress. *American Psychologist*, 44(3), 513–524. <https://doi.org/10.1037/0003-066X.44.3.513>
11. Johnson, J., Louch, G., Dunning, A., Johnson, O., Grange, A., Reynolds, C., & O'Hara, J. (2017). Burnout mediates the association between depression and patient safety perceptions: A cross-sectional study in hospital nurses. *Journal of Advanced Nursing*, 73(7), 1667–1680. <https://doi.org/10.1111/jan.13251>
12. Karasek, R. A. (1979). Job demands, job decision latitude, and mental strain: Implications for job redesign. *Administrative Science Quarterly*, 24(2), 285–308. <https://doi.org/10.2307/2392498>
13. Khamisa, N., Peltzer, K., & Oldenburg, B. (2013). Burnout in relation to specific contributing factors and health outcomes among nurses: A systematic review. *International Journal of Environmental Research and Public Health*, 10(6), 2214–2228. <https://doi.org/10.3390/ijerph10062214>
14. Kisely, S., Warren, N., McMahon, L., Dalais, C., Henry, I., & Siskind, D. (2020). Occurrence, prevention, and management of the psychological effects of emerging virus outbreaks on healthcare workers: Rapid review and meta-analysis. *BMJ*, 369, m1642. <https://doi.org/10.1136/bmj.m1642>

15. Kulkarni, A. A., Khasne, R. W., Dhakulkar, B. S., & Mahajan, H. C. (2020). Burnout among healthcare workers during COVID-19 pandemic in India: Results of a questionnaire-based survey. *Indian Journal of Critical Care Medicine*, 24(8), 664–671. <https://doi.org/10.5005/jp-journals-10071-23518>
16. Kumar, S. (2016). Burnout and doctors: Prevalence, prevention and intervention. *Healthcare*, 4(3), 37. <https://doi.org/10.3390/healthcare4030037>
17. Lai, J., Ma, S., Wang, Y., Cai, Z., Hu, J., Wei, N., Wu, J., Du, H., Chen, T., Li, R., Tan, H., Kang, L., Yao, L., Huang, M., Wang, H., Wang, G., Liu, Z., & Hu, S. (2020). Factors associated with mental health outcomes among health care workers exposed to coronavirus disease 2019. *JAMA Network Open*, 3(3), e203976. <https://doi.org/10.1001/jamanetworkopen.2020.3976>
18. Lazarus, R. S., & Folkman, S. (1984). *Stress, appraisal, and coping*. Springer. <https://doi.org/10.1007/978-1-4612-5561-1>
19. Leiter, M. P., & Maslach, C. (2009). Nurse turnover: The mediating role of burnout. *Journal of Nursing Management*, 17(3), 331–339. <https://doi.org/10.1111/j.1365-2834.2009.01004.x>
20. Maslach, C., & Jackson, S. E. (1981). The measurement of experienced burnout. *Journal of Organizational Behavior*, 2(2), 99–113. <https://doi.org/10.1002/job.4030020205>
21. Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: Recent research and its implications for psychiatry. *World Psychiatry*, 15(2), 103–111. <https://doi.org/10.1002/wps.20311>
22. Ministry of Health and Family Welfare. (2022). National health profile 2022. Central Bureau of Health Intelligence, Government of India. <https://www.cbhidghs.nic.in/showfile.php?lid=1176>
23. Pappa, S., Ntella, V., Giannakas, T., Giannakoulis, V. G., Papoutsis, E., & Katsaounou, P. (2020). Prevalence of depression, anxiety, and insomnia among healthcare workers during the COVID-19 pandemic: A systematic review and meta-analysis. *Brain, Behavior, and Immunity*, 88, 901–907. <https://doi.org/10.1016/j.bbi.2020.05.026>

24. Prasad, K., McLoughlin, C., Stillman, M., Poplau, S., Goelz, E., Taylor, S., Nankivil, N., Brown, R., Linzer, M., Cappelucci, K., Barbouche, M., & Sinsky, C. A. (2021). Prevalence and correlates of stress and burnout among U.S. healthcare workers during the COVID-19 pandemic. *EClinical Medicine*, 35, 100879. <https://doi.org/10.1016/j.eclinm.2021.100879>
25. Ramirez, A. J., Graham, J., Richards, M. A., Cull, A., & Gregory, W. M. (1996). Mental health of hospital consultants: The effects of stress and satisfaction at work. *The Lancet*, 347(9003), 724–728. [https://doi.org/10.1016/S0140-6736\(96\)90077-X](https://doi.org/10.1016/S0140-6736(96)90077-X)
26. Regehr, C., Glancy, D., Pitts, A., & LeBlanc, V. R. (2014). Interventions to reduce the consequences of stress in physicians: A review and meta-analysis. *Journal of Nervous and Mental Disease*, 202(5), 353–359. <https://doi.org/10.1097/NMD.0000000000000130>
27. Rotenstein, L. S., Torre, M., Ramos, M. A., Rosales, R. C., Guille, C., Sen, S., & Mata, D. A. (2018). Prevalence of burnout among physicians: A systematic review. *JAMA*, 320(11), 1131–1150. <https://doi.org/10.1001/jama.2018.12777>
28. Salyers, M. P., Bonfils, K. A., Luther, L., Firmin, R. L., White, D. A., Adams, E. L., & Rollins, A. L. (2017). The relationship between professional burnout and quality and safety in healthcare: A meta-analysis. *Journal of General Internal Medicine*, 32(4), 475–482. <https://doi.org/10.1007/s11606-016-3886-9>
29. Shanafelt, T. D., Hasan, O., Dyrbye, L. N., Sinsky, C., Satele, D., Sloan, J., & West, C. P. (2015). Changes in burnout and satisfaction with work-life balance in physicians and the general US working population between 2011 and 2014. *Mayo Clinic Proceedings*, 90(12), 1600–1613. <https://doi.org/10.1016/j.mayocp.2015.08.023>
30. Siegrist, J. (1996). Adverse health effects of high-effort/low-reward conditions. *Journal of Occupational Health Psychology*, 1(1), 27–41. <https://doi.org/10.1037/1076-8998.1.1.27>

31. Swensen, S., Kabcenell, A., & Shanafelt, T. (2016). Physician-organization collaboration reduces physician burnout and promotes engagement: The Mayo Clinic experience. *Journal of Healthcare Management*, 61(2), 105–127. <https://doi.org/10.1097/JHM-D-15-00021>
32. Thoits, P. A. (2011). Mechanisms linking social ties and support to physical and mental health. *Journal of Health and Social Behavior*, 52(2), 145–161. <https://doi.org/10.1177/0022146510395592>
33. Turale, S., Meechamnan, C., & Kunaviktikul, W. (2020). Challenging times: Ethics, nursing and the COVID-19 pandemic. *International Nursing Review*, 67(2), 164–167. <https://doi.org/10.1111/inr.12598>
34. Vyas, M. V., Garg, A. X., Iansavichus, A. V., Costella, J., Donner, A., Laugsand, L. E., Janszky, I., Mrkobrada, M., Parraga, G., & Hackam, D. G. (2012). Shift work and vascular events: Systematic review and meta-analysis. *BMJ*, 345, e4800. <https://doi.org/10.1136/bmj.e4800>
35. Wallace, J. E., Lemaire, J. B., & Ghali, W. A. (2009). Physician wellness: A missing quality indicator. *The Lancet*, 374(9702), 1714–1721. [https://doi.org/10.1016/S0140-6736\(09\)61424-0](https://doi.org/10.1016/S0140-6736(09)61424-0)
36. West, C. P., Dyrbye, L. N., Erwin, P. J., & Shanafelt, T. D. (2016). Interventions to prevent and reduce physician burnout: A systematic review and meta-analysis. *The Lancet*, 388(10057), 2272–2281. [https://doi.org/10.1016/S0140-6736\(16\)31279-X](https://doi.org/10.1016/S0140-6736(16)31279-X)
37. West, C. P., Dyrbye, L. N., & Shanafelt, T. D. (2018). Physician burnout: Contributors, consequences and solutions. *Journal of Internal Medicine*, 283(6), 516–529. <https://doi.org/10.1111/joim.12752>
38. World Health Organization. (2019). Burn-out an 'occupational phenomenon': International classification of diseases. WHO. <https://www.who.int/news/item/28-05-2019-burn-out-an-occupational-phenomenon-international-classification-of-diseases>

39. World Health Organization. (2020). Mental health and psychosocial considerations during the COVID-19 outbreak (WHO/2019-nCoV/MentalHealth/2020.1). WHO.
<https://www.who.int/publications/i/item/WHO-2019-nCoV-MentalHealth-2020.1>
40. World Health Organization. (2022). World mental health report: Transforming mental health for all. WHO. <https://www.who.int/publications/i/item/9789240049338>
41. Zigmond, A. S., & Snaith, R. P. (1983). The hospital anxiety and depression scale. *Acta Psychiatrica Scandinavica*, 67(6), 361–370. <https://doi.org/10.1111/j.1600-0447.1983.tb09716.x>